Clay Spine & Injury Center

5869 Nicole Shoal Apt. 796
Katy, TX
Phone 713-555-5377

Itemized Medical Bill

Patient:	Mallory Carter
Date of birth:	1957-06-28
Insurance claim:	C-1150
Date of injury:	2024-07-28
First visit:	2024-08-02
Last visit:	2025-01-01
Number of visits:	22

Services

Service	Units	Charge
99203 New patient evaluation	1	$349.86
98941 Spinal manipulation	5	$620.69
97110 Therapeutic exercise	3	$510.28
97124 Massage therapy	4	$340.43
72040 X-ray, cervical spine	3	$2,740.13
20552 Trigger point injection	6	$2,693.13
TOTAL		$7,254.52

Recent visits

Date	Service	Charge
2024-11-26	72040 X-ray, cervical spine	$983.94
2024-12-05	72040 X-ray, cervical spine	$718.58
2024-12-13	98941 Spinal manipulation	$107.77
2024-12-18	97110 Therapeutic exercise	$194.10
2024-12-26	20552 Trigger point injection	$542.24
2025-01-01	97110 Therapeutic exercise	$148.80

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Treatment History and Summary

Patient:	Mallory Carter
Treating provider:	Nancy Mcgee, DC
Diagnosis:	knee injury

Treatment history by month

Month and services	Visits	Charges
August 2024: chiropractic adjustment, evaluation, imaging, massage therapy	4	$1,595.57
September 2024: chiropractic adjustment, pain management, physical therapy	4	$886.63
October 2024: massage therapy, pain management	5	$1,504.34
November 2024: chiropractic adjustment, imaging, massage therapy, pain management	4	$1,556.49
December 2024: chiropractic adjustment, imaging, pain management, physical therapy	4	$1,562.69
January 2025: physical therapy	1	$148.80

Treatment summary

Patient presented following a low-speed collision in a parking lot with a complaint of right knee pain and reduced range of motion. The reported mechanism of injury is consistent with the force of the impact and the pattern of symptoms described. Over the course of 22 visits, care progressed along a typical rehabilitation pathway, including physical therapy, activity modification, and anti-inflammatory management. Visit frequency decreased steadily as symptoms improved and functional goals were met. Examination revealed localized tenderness and mild effusion along the medial joint line, findings that support the reported injury. Other orthopedic tests, including ligament stability assessment and neurovascular checks, were within normal limits. No prior injury or treatment involving the same body region is documented in the patient's history. Billing records reflect single, appropriate charges for each service rendered, with no duplicated entries. At discharge, the pateint was ambulating without assistance and reported minimal residual discomfort with prolonged standing.